

Cardiac Arrest Documentation

Standard:

To establish guidelines for narrating and documenting cardiac arrest care for quality improvement use.

Purpose:

The resuscitation audio recording and documentation provides a means of improving our methods, protocols, and training in order to improve the care we provide to cardiac arrest patients. The recording should describe what is happening at the scene with respect to clinical care. Providers should think of this process as equivalent to what you would say if the Medical Director were on the phone with you during the resuscitation efforts and you were describing to them what is going on at the scene.

Application:

For each cardiac arrest documentation, attempt to include as many of these elements as is possible:

1. Team leader name and Unit #
2. Witnessed arrest or not
3. Circumstances prior to arrest
4. Briefly describe the patient (age, gender)
5. Bystander CPR? If so, who?
6. Briefly describe any unusual findings
7. Patient response(s), or lack thereof, to interventions

Interventions and actions should be verbalized and documented in the ePCR:

1. Moving patient to a larger or different space
2. Compressions started/stopped, and use of metronome/feedback device
3. Switched compressors
4. AED's activation/decision to shock or not to shock, same for use of cardiac monitor
5. CPR feedback puck placed
6. Ventilation with BVM and timing light
7. EtCO₂ values and waveform
8. Placement and confirmation of BIAD
9. Pulse present/absent during AED or ECG analysis
10. Medications administered and interventions performed by providers
11. Patient has ROSC/pulses

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Crime scene management where no resuscitation is initiated:

1. Any Responder, who is not credentialed to seek a TOR of an obvious Dead on Scene (DOS), should immediately leave the area via the path of entry without touching anything.
2. When TOR is required, only one properly Credentialed Provider should make entry to the area.

Crime scene management with unsuccessful resuscitation:

1. Once resuscitation efforts have ceased and a TOR has been obtained providers should immediately vacate the area.
2. The Medical Examiner must be able to differentiate between punctures originating from resuscitation efforts and those present prior to arrival. All unsuccessful IV/IO or pleural decompression attempts should be marked on the body by circling with a marker or pen.

Crime scene management with patient transport:

1. Clothing, jewelry or other objects removed from the patient should be left on-scene. Clearly document any items left and inform on-scene Law Enforcement of the items original and current locations.
2. When cutting clothing for the purpose of assessment and/or treatment avoid cutting through existing defects in the clothing (tears, entry or exit points) whenever possible.
3. If the patient has been placed on a sheet, notify the receiving facility that the sheet and all personal effects may be considered evidence.
4. If law enforcement is not on-scene prior to transport, the first response agency is to remain on scene, out of the crime scene perimeter, until arrival of law enforcement. An effort should be made to keep all individuals out of the area.

Crime scene management with "exigent" circumstances:

1. Code of Criminal Procedure Title 1 Chapter 49.25 Removal of Bodies Section 8: "When any death under circumstances set out in Section 6 (below) shall have occurred, the body shall not be disturbed or removed from the position in which it is found by any person without authorization from the medical examiner or authorized deputy, except for the purpose of preserving such body from loss or destruction or maintaining the flow of traffic on a highway, railroad or airport."
2. In the case of these exceptions providers may be requested by law enforcement to assist with the movement/removal of the body. When possible evidence blankets should be used for patient movement and every effort should be made to preserve evidence where possible.

[Addition Information Texas Code of Criminal Procedure, Title 1. Code of Criminal Procedure, Chapter 49. Inquests Upon Dead Bodies.](#)